

Hip Injury - Female (Child)

Algorithm 1300 | Child (0-17 years) | Gender at birth: Female

Definition

- Hip injury assessment, generalized from NHS.UK's published broken arm or wrist guidance's fracture criteria.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] How did the injury happen?
2. [YES_NO] Can the person bear weight on the leg?
3. [DURATION] When did it happen?
4. [OPEN_TEXT] Record age, pregnancy, osteoporosis or frailty, anticoagulants, cancer or immune suppression, head injury, collapse before the injury, and safeguarding concern.
5. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?
6. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Motor Vehicle Accident

Did this hip injury occur as part of a motor vehicle crash or a multi-system crash injury?

A motor vehicle crash or multi-system crash injury belongs in the Motor Vehicle Accident pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

3. Disposition level 100

Is the leg shortened, rotated or deformed, is the person unable to stand or bear weight, or is there severe pain, high-energy trauma, major bleeding, numbness or weakness, a pale/cold foot, head injury, collapse, or pregnancy trauma?

Deformity, inability to bear weight, or high-energy trauma are recognized signs of a hip fracture or dislocation, consistent with the same fracture red-flag logic NHS.UK applies to limb injuries.

3. Disposition level 70

Is there significant swelling or bruising, or pain that limits walking but weight-bearing is still possible?

Ongoing pain limiting walking after a hip injury warrants prompt medical assessment to rule out a less obvious fracture.

3. Disposition level 15

Is this a minor bump or strain with none of the features above?

A minor hip strain without deformity or weight-bearing difficulty is manageable at home.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact

reporting, consent, documentation, and information-sharing rules are local approval required.

- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Emergency hip injury precautions

- Call Qatar 999 now. Do not move, straighten, or test the leg; keep the patient warm in the position found, give nothing by mouth, and do not allow self-driving. Follow the call-handler's movement and bleeding-control instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening pain, collapse, confusion, or breathing change; foot becomes pale, cold, numb, or weak.

Prompt in-person hip assessment

- Use the Qatar governance-approved in-person service and avoid weight-bearing until assessed. Older, frail, pregnant, anticoagulated, or high-risk patients need a lower threshold for direct evaluation. Confirm pain medicine with a clinician or pharmacist.
- Call Back If: worsening pain, inability to bear weight, faintness, confusion, numbness, or foot colour change.

Care for a strictly minor hip injury

- Only when walking is normal and risk factors are absent, rest from aggravating activity and use a wrapped cool pack. Confirm medication suitability and seek assessment if pain or function does not steadily improve.
- Call Back If: pain worsens, walking becomes difficult, or bruising, weakness, numbness, or illness develops.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
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References

- NHS.UK, "Broken arm or wrist" (fracture red-flag criteria), <https://www.nhs.uk/conditions/broken-arm-or-wrist/> - generalized to hip injuries

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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